



Nursing Practice Guideline

Nuclear Medicine and PET Procedures

Reference #: NPG60

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Staff

Introduction

This guideline outlines the procedures and responsibilities of individuals and the health service organisation in relation to Nuclear Medicine and PET Procedures. It ensures the delivery of a uniform, integrated multidisciplinary approach with management that is consistent with best practice.

Risk Statement

Non-compliance with this guideline will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGH Mission & Values	☒	Staff Safety	☒
Other:	☐		

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1. General Instructions

1.1 Medical Requirements

Positron Emission Tomography (PET) procedure can only be ordered by a consultant. Additional information may be obtained from the Nuclear Medicine and PET Physicians.

All patients for interventional procedures should be assessed for the ability to be positioned lying flat/still for a period of time. If the patient is unable to do so, then the referring medical officer should arrange an anaesthetic assessment prior to a booking being accepted.

Goals of Care are required for patients undergoing Myocardial Perfusion Scan- Rest / Stress studies.

1.2 Safety Information

As per procedural checklist **pregnancy status should be determined prior to procedure**. Please see individual protocol requirement regarding breastfeeding or contact the Nuclear Medicine Department for further instructions.

Patients attending PET and Nuclear Medicine (non-invasive) procedure require completion of a procedure checklist.

A nurse escort and patient handover is not required – unless clinically indicated or if the patient has been within the Senior Nurse Review criteria on the Observation and Response Chart (ORC). If the patient's clinical situation requires the nurse escort to remain for the duration of the procedure, no procedure handover checklist is required.

Note: A Nurse escort is required after hours (Mon - Fri after 1630), on weekends and Public Holidays. At these times the nurse escort will be required to remain with the patient for the duration of the procedure.

Staff should maintain standard precautions (e.g. PPE) and standard hand washing technique (i.e. handwashing with soap and water), these will protect against contamination with radioactive material.

In the event of accidental exposure to patient bodily fluids that are potentially radioactive (e.g., urine), occupational safety should take precedence, and the relevant infection control protocol should be followed first to clean the affected skin and/or eyes, which will also remove radioactive contamination. After the body fluid contamination has been removed advice should be sought from the Nuclear Medicine Department as soon as is practical; during normal business hours.

Please contact Nuclear Medicine on extension 72322 or the WA PET / Cyclotron Service on extension 72656 for further information.

As a cautionary practice it is advisable that pregnant nursing staff do not care for a patient who has undergone a PET scan within 4 hours post-PET-radiopharmaceutical-injection, which will typically be for the first two hours of their return to the ward.

It should be noted that these safety guidelines are relevant for patients who have undergone a **diagnostic** procedure in Nuclear Medicine or PET. For patients who have had therapy with a radioactive substance, specific advice will be provided by medical

physicists from either Nuclear Medicine or the Medical Technology and Physics department.

1.3 Nursing Management

- Prior to patient transfer ensure Incontinence pads are changed and IDC bags to be empty.
- Clothing Requirements:
 - No metal, patient need to remove jewellery, bras etc.
 - Patients must leave all valuables on the ward.
- Medical / nursing notes must accompany the patient to the Department.
- If an interpreter is required, this must be booked by the referring ward.
- If the patient cannot give consent, it must be arranged with the next of kin before the imaging/procedure can proceed.
- Review pain relief management, as imaging procedure requires that patient's lie still often for long periods of time (in excess of 2 hours) on the camera bed (which is a hard metal surface).
- Radioactive isotopes covered in these guidelines are administered in the department for diagnostic purposes (patients experience no side effects from these substances).
- There are specific guidelines for patients receiving therapeutic doses.
- Nurse caring for patient to inform patient, family and carer (if present) that the time the patient is away from ward will be longer than camera time hours stated. This is due to transport and waiting times in the department.

Any clinical information enquiries for patients attending the following areas please call appropriate extension:

- Nuclear Medicine extension 74343, or
- PET extension 77314

2. Nuclear Medicine Studies

STUDY	CAMERA TIME AND HOURS IN DEPARTMENT	INSTRUCTIONS
ADRENAL MIBG ¹²³ I or ¹³¹ I For diagnosis of Phaeochromocytoma, Medullary Hyperplasia or Neuroblastoma	Patient will be in department for up approx. 2 hours on both days. Day 1 – scan time 30 mins Day 2 – Scan time 2 hours With camera time on day 1 lasting 30 mins and day 2 lasting 2 hours	Injection on day of scan, with further scan on day 2. 1 ml of Lugols Iodine to commence one hour before the injection. Interruption to breast-feeding for 22 hours post-procedure.*
BLOOD VOLUME (Red Cell Mass and Plasma Volume) Investigation of polycythaemia	Patient will be in the department for approx. 1 ½ hours. Nil imaging	IV Access (18 gauge) Separate IV access is required if Continuous IV Therapy is in progress

BONE ^{99m}Tc HDP To evaluate symptoms related to the skeleton, investigate a wide variety of conditions and to demonstrate areas of increased osteoblastic activity in bone tumours	Patient will be in the department for approx. Part 1 – injection – patient will be in department 15-30 minutes with possible scan 15 minutes. Part 2 – Bone Scan – patient will be in the department for 1-2 hours with a scan lasting 1.5 hours.	IV Access Injection 2-5 hours prior to scan Encourage oral fluids between injection and scan Pain relief should be given 30 minutes prior to delayed scan Ensure patient has voided prior to attending 2nd imaging appointment. Patients should also be encouraged to empty their bladder immediately before delayed scan. Incontinent patients to have peri-wash and fresh pad in-situ Interruption to breast-feeding for 2 hours post-procedure.*
BONE MARROW ^{99m}Tc Antimony Colloid To demonstrate orthopaedic infection by correlation with White Blood Cell study	Patient will be in the Department up to approx.2 hours. Scan time- - 1 hour	IV Access Injection ½ hour pre scan
BRAIN DEATH with ^{99m}Tc-ECD Confirmation for determination of brain death	Time in Department approx. 1 hour Camera time - 1 hour	IV Access Liaise with Nuclear Medicine department Ext 74343 to arrange the camera time.
BRAIN DIAMOX STUDY DIAMOX with ^{99m}Tc-ECD To evaluate cerebral flow reserve using Diamox for cerebral artery vasodilation	Patient can be in the department up to 3 hours. Camera time - 1 hour	IV Access required in each arm Baseline measurements of blood pressure and pulse rate required Patient imaged 1-2 hours post injection Interruption to breast-feeding for 12 hours post-procedure.*
BRAIN with ^{99m}Tc-ECD To establish cerebral perfusion	Patient can be in the department up to 2-3 hours. Camera time - ½ hour	IV Access Patient has 20 mins rest and is injected in a low stimulus environment, Total procedure time 1½ - 2 hours

		Interruption to breast-feeding for 12 hours post-procedure.*
BRAIN EPILEPSY ^{99m} Tc Ethyl Cysteinate Dimer (ECD) Pre-surgical localisation of epileptogenic region in intractable epilepsy	Time in Department approx. 1 hour Camera time – 30mins	Dose is administered via cannula as soon as possible following seizure onset by trained nursing staff Imaging should be completed within 4 hours post injection if possible Neurology staff escort and remain with patient during imaging. Interruption to breast-feeding for 12 hours post-procedure.*
CARDIAC LVEF – MIBI EJECTION FRACTION ^{99m} TcSestamibi Assess the amount of blood pumped out of the left ventricle with each beat.	Patient could be in department approx. 1.5 hours. Camera time - ½ hour	IV Access Patient must have no chest pain for 24 hours Patient injected 1 hour prior to scan. Interruption to breast-feeding for 1 hour post-procedure.*
COLONIC TRANSIT ⁶⁷ Ga Citrate Evaluation and clinical diagnosis of constipation	Patient in department for 30 min multiple appointments over several days Camera time - ¼ hour	The dose is administered orally in a cup of water. No laxatives for 3 days before and during study. Medications which affect gastric motility should be ceased (e.g. morphine). Light breakfast on day of study. The patient is told to eat a normal lunch and to return in 6 hours' time. Rescan Days 1 - 4 and Day 6 if necessary
GASTRIC BLEED ^{99m} Tc-RBC To identify site of intermittent internal bleeding	Time in department approx. 2 hours Part 1 - Blood sample Part 2 - Scan can take up to 1½ hours.	IV Access (18 gauge) Interruption to breast-feeding for 12 hours post-procedure.*
GALLIUM BONE or INFECTION ⁶⁷ Ga Citrate Whole body survey to localised source of infection	Time in department Part 1 – Injection can take 15 mins-20 minutes.	IV access for Part 1 Scan completed 48 hours post injection Cessation of breast-feeding.

	Part 2 – Scan ½ -- Camera time approx. 1½ hour	
GALLIUM LUNG ⁶⁷ Ga Citrate Quantitative survey of Gallium uptake	Time in department Part 1 – Injection can take 15-20 minutes. Part 2 – Scan can take approx. 35 mins	IV Access Patient must have recent Chest X-ray available prior to procedure Injection 48 prior to scan <u>Cessation of breast-feeding.</u>
GASTRIC EMPTYING SOLID & LIQUID ^{99m} Tc Pertechnetate To evaluate the half-emptying time of the solid phase of gastric emptying or To evaluate gastric emptying of patients unable to tolerate a solid meal	Time in Department approx. 3-4 hours Camera time 1-2½ hours Patient can have multiple scans with possible additional 15min delay scans throughout the day.	IV Access Fasting: 4 hours for liquid study 8 hours for solid study No smoking or coffee/ tea on the morning of the scan BSL must be below 12mmol/L for diabetics Laboratory needs ½ hour notice to prepare meal Inform Nuclear Medicine Department if patient has any dietary requirements or food allergies. Medications to be withheld for 2 days: • prokinetic agents (e.g., metoclopramide, erythromycin, domperidone), or • medications that delay gastric emptying – opiates, antispasmodic agents, anticholinergics, nifedipine, benzodiazepine Patient to consume the meal prepared by lab staff over 5 minutes, alternating solid and liquid. Finish with liquid.
HEPATIC ARTERY PERFUSION / SIRT WORKUP ^{99m} Tc MAA To demonstrate hepatic arterial perfusion of the liver. Done as pre-workup for SIRT sphere therapy.	Patient could be in the Department up to approx. 1 hour Scan time -¾ hour	Patients will attend Radiology Day Unit for the first phase of this procedure (for insertion of hepatic artery catheter under fluoroscopy guidance). Liaise with Nuclear Medicine department Ext 74343 to arrange the camera time. Patient will then be transferred to Nuclear Medicine for imaging. Interruption to breast-feeding for 13 hours post-procedure.*

HEPATOBIILIARY ^{99m} Tc DISIDA For confirmation or exclusion of acute cholecystitis To determine presence of chronic cholecystitis and calculate gall bladder ejection fraction	Time in department Acute - 1½ - 2 hours Chronic 2-½ - 3 hours	IV Access Avoid opiates Fast between 6-12 hours (do not fast longer than 12 hours) Consent for administration of CCK is obtained in the Nuclear Medicine department. CCK is administered to cause contraction of the gallbladder
VQ (LUNG) ^{99m} Tc Technegas and ^{99m} Tc MAA To demonstrate the presence of Pulmonary Embolus Study done in 2 parts, the inhalation of the technegas to evaluate ventilation to the lungs and the IV administration of the isotope to evaluate the perfusion of the lungs	Patient will be in the department approx. 1.5 hours Scan time – ¾ hour Pt needs to lie flat for scan duration	IV Access Patient positioned supine Patient is to remain in department until medical review completed Patient must have recent Chest X-ray available prior to procedure Interruption to breast-feeding for 13 hours post-procedure.*
LUNG DIFFERENTIAL ^{99m} Tc MAA To assess perfusion of the lobes of the lungs in patients being considered for, and post, lung resection surgery and lung transplant	Patient will be in the department approx. 1.5 hours. Scan time — ½ hour Pt needs to lie flat for scan duration	Perfusion only study IV Access Interruption to breast-feeding for 13 hours post-procedure.*
LUNG SHUNT ^{99m} Tc MAA To demonstrate A-V shunt with quantitation	Patient will be in the department for approx. 1.5. hours Scan time - ¾ hour	Patient must be able to stand for >10 minutes for this scan. Notify Department of any mobility issues. IV Access Interruption to breast-feeding for 13 hours post-procedure.*
FUTURE LIVER REMNANT ^{99m} Tc-Mebrofenin	Patient will be in the department for approx. 1 hour. Scan time - ½ hour	Patients' height and weight required. Same-day diagnostic CT scan required.

Preoperative evaluation of future remnant liver (FRL) is crucial in the determination of whether a patient can safely undergo liver resection		
BREAST SNS/LYMPHOSCINTIGRAPHY ^{99m} Tc Sb Colloid To assess lymphatic drainage from breast tumours and to identify Sentinel Lymph node(s), especially in the axilla	Patient will be in the department for approx. 1 hour. Scan time - 1/2 hour	Hospital Gown Consent to be completed by referring team Scan commences 5-15 minutes after injection, may require delayed views
LYMPHOEDEMA SNS/LYMPHOSCINTIGRAPHY ^{99m} Tc Antimony Sulphur Colloid Evaluation of lymphoedema of the lower extremities Isotope is injected intradermally between the toes	Time in department approx. 2 hours Camera time - 1 hour & ¾ hour delayed scan (if required)	Hospital Gown Scan commences at injection with delayed views at 2 and 2½ hours Isotope injected intradermal between the toes
MELANOMA SNS/LYMPHOSCINTIGRAPHY ^{99m} Tc Antimony Sulphur Colloid To demonstrate lymphatic drainage from melanoma The Isotope is injected intradermal around the periphery of the lesion or surgical site.	Time in department approx. 2 hours Camera time - ½ hour at injection and possible 1 hour at 3 hours post injection	Hospital Gown Consent to be completed by referring team Scan commences at injection with delayed views at 2 and 2½ hours
MECKELS DIVERTICULUM ^{99m} Tc Pertechnetate To demonstrate presence of ectopic gastric mucosa	Time in department approx. ½ hour at time of injection Re-imaged at 1 hour post injection	IV Access Treating medical staff to prescribe: Omeprazole (E.g. Losec) 40mg day prior to scan and 40mg morning of scan. Post scan patient must not breast-feed for 34 hours.
STRESS TEST/MYOCARDIAL PERFUSION STUDY Two Day Protocol ^{99m} Tc Sestamibi Adenosine, Exercise or Dobutamine Detection of coronary artery disease, evaluation of resting chest pain, documentation of myocardial	Part 1 – REST patient will be in the department approx. 2 hours. Scan time -with ½ hour camera time.	IV ACCESS First appointment – Rest study

perfusion abnormality before and after interventional therapy Stress study may involve an infusion of Adenosine/Dobutamine or exercise (treadmill)	Part 2 – STRESS patient will be in the department approx. 3 hours Scan time -with ½ hour camera time. Time in department may increase if we need additional scans	Second appointment – Stress Study. Cardiac Scan Checklist to completed prior to rest study For Stress Study: Goals of Care to be reviewed by Medical Team if patient NFR or NFM 4 hour fast NO CAFFEINE FOR 24 HOURS PRIOR TO STRESS TEST Medical staff to withhold: Dipyridamole medication 48 hours pre stress test, all xanthine type medications (e.g. Theophylline) for at least 24 hours pre dipyridamole study. REMOVE GTN: Patches 24 hours pre scan. GTN IV infusion must be stopped 4 hours pre scan. Oral GTN– continues if necessary. Interruption to breast-feeding for 4 hours post-procedure.*
RENAL CAPTOPRIL ^{99m} Tc MAG3 To determine the presence of renal artery stenosis	Time in department approx. 2 hours Camera time 45mins	Patient to have light breakfast at 0600 hours and then fast from solids, Water only from 0600hr IV Access (20 gauge) Medical staff to confirm with nuclear medicine physician if antihypertensive medication needs to be withheld. Appointments in am and 1½-hour prior to scan. In department: Patient is given 1 - 1.5 litres of fluid to drink over the hour
RENAL DMSA ^{99m} Tc DMSA (dimercaptosuccinic acid) High resolution imaging of the renal cortex	Time in department Injection – 15 minutes Camera time - ½ hour	IV Access Hydration is NOT necessary for the DMSA study. Patient is injected with the ^{99m} Tc DMSA 3 hours prior to imaging
RENAL FUNCTION ^{99m} Tc MAG3 For renal function and perfusion or PLUS DIURESIS ^{99m} Tc MAG3 renal perfusion, function & obstruction	Time in department 1 -2 hours Camera time - ½ -¾ hour	IV Access Encourage oral fluids 45mins pre-scan Full bladder not required IV diuretic administered 15 mins pre-scan in Nuclear Medicine

RENAL GFR ^{99m} Tc DTPA To accurately assess renal differential function	Time in department approx. 4 hours	Patient to have light, low protein meal. IV Access Blood is taken and labelled with ^{99m} Tc DTPA The labelled blood is injected back into the patient A blood sample is taken every hour for 3 hours after injection.
RENAL TRANSPLANT ^{99m} Tc MAG3 To assess kidney transplant perfusion, output and function	Time in department approx. 1 hour Camera time - ¾ hour	IV Access
SPLEEN ^{99m} Tc Invitro Heated Damaged RBCs To diagnostically visualise the spleen, usually looking for local lesions indicating trauma of infarction	Time in department Part 1 Blood sample – ½ hour Part 2 - Camera time - ¾ hour	IV Access Patient's blood is withdrawn, labelled with isotope and returned to the patient Patient imaged 2 hours post blood returned
SPLENUNCULI ^{99m} Tc Labelled Autologous Red Blood Cells To demonstrate the presence of splenunculi	Time in department Part 1 – Blood sample – ½ hour Part 2 – Camera time - ¾ hour	IV Access (18 gauge) Patient's blood is withdrawn, labelled with isotope and returned to the patient Patient imaged 2 hours post blood returned
THYROID FUNCTION ^{99m} Tc Sodium Pertechnetate Assessment of overall function and structural status of the thyroid	Time in department for approx. 1 hour Camera time - ½ hour	IV Access No contrast media (e.g. IVP, CT scan, angiogram) for 6 weeks prior and eat a low iodine diet 2 weeks prior to imaging (Contact Nuclear medicine department Thyroid Nurse for instructions) Interruption to breast-feeding for 26 hours post-procedure.*
THYROID REMNANT (DRINK) Iodine-123 Drink For detection of thyroid remnant, tumour and metastases	Time in department Drink – 30 mins Camera time - ¾ hour	Dose drink administered by laboratory 24 hours prior to scan Interruption to breast-feeding for 21 hours post-procedure.*

THYROID REMNANT (CAPSULE) ¹³¹ I as Sodium Iodide (capsule) Post ¹³¹ I therapy check for detection of thyroid remnant and functioning metastases	Time in department approx. 2 hours Camera time - 1½ hours	Capsule administered by Nuc Med 48 hours prior to scan. <u>Cessation</u> of breast-feeding.
WHITE BLOOD CELLS ^{99m} Tc HMPAO Infection including Osteomyelitis. To determine presence of infection, acute and sub-acute, in organs other than lung, liver and kidney & Pyrexia of unknown origin To determine presence of acute or sub-acute infection in organs other than lung, liver, kidney, orthopaedic	Time in department Part 1 – Blood sample taken approx. 15 mins Camera time – scan at 1 hour and 4 hours	IV Access (18 gauge) Patient to arrive in dept. 2½ hour pre injection time. 60ml of patients' blood is withdrawn, labelled with isotope and returned to patient Scan at 1 and 4 hours post injection (total time in dept is up to 4 hours) Interruption to breast-feeding for 21 hours post-procedure.*

3. PET Procedures

STUDY	CAMERA TIME AND HOURS IN THE DEPARTMENT	INSTRUCTIONS
18F-FDG BRAIN SCAN Diagnosis of disorders such as Alzheimers and fronto-temporal dementia. Pre-operative evaluation of epilepsy.	Time in the department approx. 3 hours Camera time - 20 minutes	IV Access 4 hr fast Interruption to breast-feeding for 1 hour post-procedure.*
18F-FDG WHOLE BODY SCAN (For lung, colorectal, lymphoma, melanoma, sarcoma, oesophageal, ovarian, glioma and, Infection) Patient injected with FDG via cannula, rest 60 minutes then imaged.	Time in department approx. 3 hours Camera time - 15-45 minutes	IV Access Fast from solids for 6 hours. A light meal (low carbohydrate/sugar free) can be consumed prior to the fasting period, water only up until time of procedure Diabetics – all medications to be given before 6 hours prior to appointment with light meal. Please call nursing staff if patients BSL >11mmol/L Parental nutrition and intravenous fluids containing glucose should be discontinued at least 6 hours prior to FDG administration Insulin pump kept on "night setting" until after the PET scan (i.e. for at least 4 hours prior to appointment). All incontinence pads to be changed prior to scan.

		Patients on peritoneal dialysis need to be dialysed 6 hrs prior to appointment Sedation if required, needs to be arranged by team Interruption to breast-feeding for 2 hours post-procedure.*
18F-FDG CARDIAC VIABILITY SCAN Differentiate areas of scarring from chronically ischaemic but viable myocardium	Time in department 3.5 hours Camera time- 15-45 minutes	Patients have a light breakfast or lunch prior to appointment (no fasting required) IV Access BSL>20 mmol/L = inform referring physician & rebook patient Medical staff to review medications, note: <u>Diabetic Patient</u> takes half their usual medication (insulin or oral meds) prior to PET scan with meal. Acipimox is administered in the PET Dept. See post-care instruction. Interruption to breast-feeding for 1 hour post-procedure.*
18F-FDG CARDIAC SARCOID To assess inflammatory conditions of the myocardium or pericardium. <u>Suspected conditions</u> Myocardial: Sarcoidosis, myocarditis, pericarditis, and epi-pericarditis Patient injected with FDG via cannula, rest 60minutes then imaged.	Time in department 3.5 hours Camera time -15-45 minutes	IV Access 12-hour fast diet and fluids 24-hours prior to the study, commence high fat diet and avoid sugar in any form. See Dietary Instruction Sheet Parenteral nutrition and intravenous fluids containing glucose should be weaned as per Nursing Practice Guideline No. 7 –Total Parenteral Nutrition, to ensure TPN is discontinued at least 4 hours prior to FDG administration <u>Diabetics</u> – diabetic medications withheld day of scan Unfractionated Heparin (50IU / kg IV) at 15 minutes prior to injection of FDG. Interruption to breast-feeding for 2 hours post-procedure.*
68GALLIUM OCTREOTATE SCAN Somatostatin receptor avidity in Neuroendocrine tumours (NET's)	Time in department approx. 3.5 hours Camera time -15-45 minutes	IV Access No fasting required Patient is imaged 1 hour post injection Interruption to breast-feeding for 2 hours post-procedure.*
68GALLIUM PSMA SCAN To assess for malignant prostate tissues	Time in department approx. 3.5 hours	IV Access No fasting required Patient is imaged 1 hour post injection

	Camera time - 15-45 minutes	
18F-PSMA SCAN To assess for malignant prostate tissues	Time in department approx. 4 hours Camera time – 15-45 minutes	IV Access No fasting required Patient is imaged 2 hours post injection
18F- DOPA To diagnose Parkinson's disease vs other neurodegenerative diseases with motor dysfunction Also used for thyroid cancer and insulinoma (no carbidopa given)	Time in department approx. 3 hours Camera time 15-45 minutes	IV Access Low Protein Diet from midnight 2 hours Fast Carbidopa 200mg given 1 hour prior to injection.

*** For breast-feeding interruptions, during the withholding time the patient can express breast milk and freeze the milk. Then, after the withholding period has ended, this milk can be used.**

Related Documents

Legislation

- Health Services Act 2016
- Work Health and Safety Act 2020
- Occupational Safety and Health Regulations 1996

Policy:

- NMHS Policy Occupational Safety and Health Personal Protective Equipment
- NMHS Occupational Safety and Health Incident Hazard Reporting and Investigation
- NMHS Clinical Documentation Policy
- SCGOPHCG Patient Identification
- SCGH Procedure Matching
- SCGOPHCG Clinical Handover-Communicating for Safety

Bibliography

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Authorisation & Version Control

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For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

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